

STROKE NURSING CARE PATHWAY

King Fahad Central Hospital

Stroke Nursing Care Pathway

Emergency Arrival Through Discharge — Aligned with AHA/ASA Get With The Guidelines—Stroke and American Stroke Association

Document Code: SU-006-PATH-01	Version: 1.0
Parent Policy: SU-006 - NIHSS Assessment, Neurological Monitoring and Dysphagia/Swallow Screening	Master Manual: SU-MANUAL-01

International Stroke Center nursing requirements

1. Purpose

This pathway outlines nursing assessment, intervention, and documentation expectations across the stroke care continuum — from prehospital notification/ED arrival through discharge — to support consistent, guideline-concordant care and American Stroke Association International Stroke Center certification and AHA/ASA quality-measure readiness.

2. Phase 1 — Emergency / Hyperacute (Arrival to Disposition)

2.1 Immediate Nursing Actions on Arrival

- ☐ Confirm last known well (LKW) time; determine IV thrombolysis / thrombectomy eligibility window
- ☐ Perform rapid stroke screen (e.g., Cincinnati Prehospital Stroke Scale, FAST-ED) and activate stroke alert/code stroke per protocol
- ☐ Obtain full vital signs, weight (actual measured, not estimated), and fingerstick glucose immediately
- ☐ Establish 2 peripheral IV lines
- ☐ Obtain 12-lead ECG
- ☐ Draw STAT labs per protocol (CBC, BMP, coagulation studies, troponin, type and screen as ordered)
- ☐ Assist with/document NIH Stroke Scale (NIHSS)
- ☐ Expedite non-contrast head CT / CTA per door-to-imaging time target
- ☐ Keep patient NPO until formal dysphagia screen completed
- ☐ Notify stroke team/physician of all results immediately; document times for door-to-CT, door-to-needle, door-to-groin-puncture metrics
- ☐ If IV thrombolysis ordered: perform independent double-check of weight-based dose per Alteplase Preparation & Administration Protocol
- ☐ Facilitate transfer to angiography suite without delay if thrombectomy indicated

2.2 Family/Patient Communication (Hyperacute Phase)

- ☐ Identify and orient family/support person as soon as feasible; designate communication contact
- ☐ Provide brief, honest updates on plan of care and expected next steps
- ☐ Facilitate informed consent process for thrombolysis/thrombectomy per institutional policy

3. Phase 2 — Acute Inpatient Care (First 24–48 Hours)

3.1 Monitoring & Assessment

Parameter	Frequency / Nursing Action
Neuro checks + vital signs	Per post-tPA/post-thrombectomy protocol if applicable (e.g., q15min ×2h, q30min ×6h, q1h ×16h); otherwise q1–2h per unit protocol — notify physician for NIHSS increase ≥2 points, new deficit, decreased LOC
Cardiac monitoring	Continuous telemetry ≥24–48h to detect atrial fibrillation
Pulse oximetry	Continuous; supplemental O2 to keep SpO2 >94%
Blood pressure	Per physician-ordered parameters — do not treat outside ordered thresholds without notifying physician
Temperature	Monitor q4h; treat fever >38.0°C with antipyretic per order — maintain normothermia
Blood glucose	Fingerstick per protocol (e.g., q6h); treat hyper/hypoglycemia per protocol, target 140–180 mg/dL
Intake/output	Strict I&O; monitor for urinary retention

3.2 Dysphagia & Nutrition

- ☐ **Maintain NPO (including oral medications) until formal bedside dysphagia screen completed by trained nurse/SLP — required before ANY oral intake**
- ☐ Refer to Speech-Language Pathology if screen failed or NIHSS/exam findings concerning
- ☐ Advance diet per screen result and physician order; reassess swallow status with any change in neuro status
- ☐ Aspiration precautions: HOB ≥30°, supervised feeding, thickened liquids if ordered

3.3 Positioning, Skin & Mobility

- ☐ HOB elevated ≥30° once medically stable
- ☐ Reposition/turn q2h; perform skin assessment each shift; initiate pressure-injury prevention measures for immobile patients
- ☐ Apply sequential compression devices for VTE prophylaxis while immobile; administer pharmacologic prophylaxis per order once hemorrhage excluded
- ☐ Coordinate with Physical/Occupational Therapy to initiate tiered early mobilization per institutional Early Mobilization Protocol once safety screen passed
- ☐ Fall risk assessment each shift; implement fall precautions (bed alarm, non-slip footwear, call light in reach, assistance with transfers)

3.4 Bowel & Bladder

- ☐ Assess for urinary retention; avoid indwelling catheter unless clinically necessary — remove as soon as possible to reduce catheter-associated UTI risk
- ☐ Bowel regimen per order; monitor for constipation, especially with reduced mobility

3.5 Medication Administration

- ☐ Administer antithrombotic, statin, antihypertensive, and other medications per physician order and timing windows (e.g., hold antithrombotic until 24h post-tPA and hemorrhage excluded)
- ☐ Perform medication reconciliation on admission; verify home medications held/resumed per physician review
- ☐ Monitor for adverse effects; document administration times accurately for core-measure reporting

3.6 Communication, Cognition & Psychosocial Support

- ☐ Assess for aphasia, dysarthria, neglect, or cognitive changes; adapt communication approach (visual aids, yes/no questions, allow extra time)
- ☐ Screen for post-stroke depression/emotional distress per institutional tool; refer to social work/psychology as indicated
- ☐ Involve family/caregiver in care planning and education as appropriate
- ☐ Provide emotional support; validate patient/family fear and uncertainty without over-reassuring on prognosis

4. Phase 3 — Ongoing Care & Discharge Preparation

4.1 Rehabilitation Coordination

- ☐ Confirm PT, OT, and Speech-Language Pathology evaluations completed within 24–48h of admission
- ☐ Communicate mobility tier and functional status changes to multidisciplinary team each shift
- ☐ Participate in interdisciplinary rounds/discharge planning meetings

4.2 Patient & Caregiver Education (document completion in chart)

- ☐ Stroke warning signs and F.A.S.T. — when to call 911
- ☐ Individualized risk factor modification: blood pressure control, diabetes management, cholesterol, smoking cessation, physical activity, diet/sodium
- ☐ Purpose, dose, and importance of adherence for each discharge medication, including antithrombotic/anticoagulant bleeding precautions if applicable
- ☐ Signs of bleeding or stroke recurrence to report
- ☐ Follow-up appointments scheduled and communicated (Neurology/Stroke clinic, PCP, rehabilitation)
- ☐ Home safety and fall-prevention guidance if mobility deficits persist
- ☐ Written stroke discharge instructions provided in patient's preferred language
- ☐ Smoking cessation counseling/resources offered if applicable
- ☐ Caregiver training on assistance with mobility, swallowing precautions, and medication administration if needed

4.3 Discharge Readiness & Case Management

- ☐ Case Management/Social Work engaged for discharge disposition planning (home, inpatient rehab, skilled nursing facility)
- ☐ Confirm equipment/home health/outpatient therapy referrals in place as needed
- ☐ Confirm transportation and support system for discharge
- ☐ Reconcile and provide discharge medication list; confirm prescriptions sent/filled
- ☐ Modified Rankin Scale or functional status documented at discharge for registry reporting

5. Nursing-Sensitive Quality & Core Measures Checklist

- ☐ Dysphagia screen completed before any oral intake
- ☐ VTE prophylaxis initiated by end of hospital day 2
- ☐ Antithrombotic therapy given by end of hospital day 2
- ☐ Stroke education documented as provided to patient/caregiver
- ☐ PT/OT/SLP evaluations completed within 24–48h
- ☐ Assessed for rehabilitation services
- ☐ Time to first mobilization documented
- ☐ Door-to-CT, door-to-needle, and/or door-to-puncture times documented accurately
- ☐ Discharge follow-up plan documented

6. Nursing Competency & Education Requirements

Reflects typical Joint Commission Primary/Comprehensive Stroke Center expectations — confirm exact hours/frequency against current certification standard.

- ☐ Stroke-specific nursing education completed on hire/orientation and annually thereafter, per institutional requirement
- ☐ NIHSS certification current for nurses performing stroke scale assessments
- ☐ Dysphagia screening competency validated for nurses performing bedside swallow screens
- ☐ Participation in stroke case review / performance improvement activities as assigned

Nursing Documentation Sign-Off

RN Signature: _____ Date/Time: _____

Unit: _____ Shift: _____

References: AHA/ASA Get With The Guidelines–Stroke program measures; 2019 AHA/ASA Guidelines for the Early Management of Patients With Acute Ischemic Stroke and subsequent focused updates; The Joint Commission Primary and Comprehensive Stroke Center certification requirements, including nursing education and competency standards; institutional Early Mobilization and Alteplase Administration protocols. This template should be re-verified against the current guideline and certification standard version at the time of institutional adoption.